

# Chapter 17

# Working with Specific Populations

Group leaders often work with specific populations. For each population, there are some unique leadership considerations. In this chapter, we have selected 11 populations that we feel present some special challenges. We realize that there are a number of other populations that we could address, but we think these are among the most frequently led groups or populations. The purpose of this chapter is to explore some of the group counseling issues concerning these 11 selected populations. It is important for you to understand that this chapter is meant only as an overview to these specific groups and that you would want to read much if you were asked to develop a group program for these or any other population. An excellent resource for books on groups with various populations is the Self Esteem Shop (www.selfesteemshop.com). This Web page is an excellent place to start if you don't already have some favorite Web sites for resources.

In this chapter, we discuss mainly support, growth, counseling, and therapy groups within each of the following populations.

1. Children
2. Adolescents
3. Couples
4. Addiction groups
5. Older clients
6. Clients with chronic diseases or disabilities
7. Survivors of sexual abuse
8. Divorce groups

9. Adult children of alcoholics
10. Multicultural groups
11. Military issues

In this edition, we have added a section on the issues that pertain to current military and veterans and their family members. Given our current military involvement in the world, we added this because we feel many of you will have the opportunity to lead these groups.

## Children

For this book, we define children to be under the age of 12. Groups for children take place mostly in schools, although some agencies conduct groups for children dealing with specific concerns. According to Shechtman (2004), "There is a consensus among professionals, based on research evidence, that group intervention with children is effective" (p. 7). School groups are conducted by the school counselor or by a counselor from a mental health center who is assigned to the school. Most of the groups for children deal with family issues, values, self-concept, building social skills, and building academic skills.

For some children, groups can be much better than individual counseling because groups allow children a place to learn and practice new skills. Counseling and support groups for children are extremely valuable—children get to hear that they are not alone in their feelings. Groups also provide good information, support, and counseling for children living with an alcoholic, children whose parents are going through a divorce, children who are beginning to live in a stepfamily, or children who are facing other difficult situations. For groups with young kids (below the age of 7), we suggest that much of the group time be spent in play time (drawing, toys, sand tray) with some sharing and processing, or in group discussion regarding a book that is read to them. Effective groups have been conducted with 4- or 5-year-old children, where they play with one another and talk about their play.

There are now many excellent, helpful resources for anyone who is leading groups for children. In the exhibit area of any state or national counseling conference, one can find more than 30 books that have activities and ideas for leading children's groups. If you are just getting started, seek out material that can help you with the specific groups that you are leading. If you are planning to lead groups with children, we suggest that you read either Rosemarie Smead's book *Skills and Techniques for Group Work with Children and Adolescents* (1995) or Kenneth Greenberg's *Group Counseling in K–12 Schools* (2003), in which they describe setting up and conducting groups with children. They also discuss such aspects as working with principals and teachers, obtaining permission from parents, confidentiality, and how to prepare for the group. Ann Vernon (2010) provides another outstanding book for children, *More What Works with Children and Adolescents,* with activities coming primarily from an REBT perspective.

Confidentiality with groups of children and adolescents has to be understood by any group leader. School counselors should become familiar with the American School Counselor Association's ethical standards for their stance on confidentiality in school groups. In most schools and treatment settings for children and adolescents, files and case notes are open to parents, so confidentiality is limited.

## Kinds of Groups

Many different groups are available for children. They often deal with specific topics or on skill development. The following is a list of some of the groups that are currently being led in schools.

Self-concept or self-esteem
Anger management
Getting along with siblings
Shyness
Friendship
Divorce
Living in a stepfamily
Living with a chemically dependent parent
New-to-the-school
Dealing with bullies
Dealing with name calling
Grief support (after some tragic event)
Study skills building
Building social skills
ADHD children
Improving academics

The following issues are addressed in some of the agency-led groups for children:

Severe behavior problems
Divorce
Alcoholic parents
Abused children (upper ages; need a very skilled leader)

## Screening

Leaders must carefully screen in groups for children because a group situation is very difficult for some children. Often teachers serve as an excellent referral source. However, some referrals from teachers or self-referrals from children are not always appropriate for groups. Individual counseling may better serve children who demand an inordinate amount of attention, who just can't seem to be able to focus for any length of time, or who can't share time well with others.

Leaders should think of the first session of a children's group as a time for screening. Children should be told that the first meeting is for learning how they like being in a group and how they do in a group situation. (See *Screening* in the index for additional information.)

## Group Size

Children's groups are usually smaller than adolescent and adult groups because the attention span of a child needs to be considered. When there are more than six members, it is hard for most children to stay focused. Four is a good number, but often one or more are absent, and then the group is small. For groups where the members have trouble paying attention or interacting with others, the leader may want to have a maximum of four children. A group size of three children in an ADHD group or a group with severe behavior problems is common.

## Length of Session

Most children's groups last around 30 minutes, although some may be as long as 40 minutes or as short as 20 minutes. Each leader has to decide what length is the best, given the ages of the children, the topic or purpose, and the members in the group.

## Number of Sessions

Some groups for children last for only one or two sessions because they are more psychoeducational or deal with a specific topic that can be covered in a couple of sessions. Other groups may last for eight or more sessions, such as dealing with divorce, making friends, or living in a stepfamily. School counselors may want to structure the number of sessions around the number of weeks in the school term or quarter.

## Special Skills

There are a number of important skills that are necessary for leading groups with children.

*Use a leader-oriented model as opposed to a facilitator model.* One of the main differences in leading groups with children is that the leader usually needs to take more responsibility for the group than in a group composed of adults. Picking good books and activities is a must for children, because they do not usually come prepared to discuss any topic. There are numerous books on many subjects that may arise in counseling for children. Search catalogs and bookstores, and ask other experienced counselors for material on subjects you want to cover in a group.

*Vary the format and use a multi-sensory approach.* Exercises, short stories, skits, and the use of puppets, drawing, and other props all are helpful tools for the leader. If the proper tone is established, children usually are more than eager to talk.

*Cut off and hold the focus.* A primary skill in leading children's groups is holding the focus on a given topic long enough so that there is some impact. In fact,

the leader may frequently need to cut off members to let others share. Often, other children will change the subject and the leader will need to bring the group back to the topic of discussion. Leaders have to be prepared to gently redirect discussion and not let a member drift into other topics before completing the first discussion.

*Use an enthusiastic voice.* When working with children, the leader's use of his voice can be an effective tool. It is very important to show enthusiasm and excitement about the subject being discussed. This gets the children interested and focused. Leaders who have a dull voice rarely are able to have successful groups with children.

*Clarity of purpose—get children to think, but understand that much of the counseling will be done privately.* In workshops for elementary school counselors, participants have commented that they had too much "fluff" (fun and games) in their groups and not enough depth. Granted, with young children it is sometimes hard to take the group deeper, but if the leader is clear about the group's purpose, she can have a productive session. By using props, creative ideas, and good leadership skills, the focus can be held and taken to a level that is meaningful for the members. Usually the counselor will not do individual work for any length of time, because the other children would probably become restless, but the focus on a topic can go to a 7 or a 6 level on the depth chart. Often, a group session can lead to an individual follow-up session.

*Special knowledge.* For each different kind of group, the leader needs not only group leadership skills but also knowledge of specific topics such as the effects of divorce on children, problems when entering a new stepfamily, how to improve school performance, or living with a chemically dependent person.

## Comments from Practicing Counselors

We asked four counselors who work with children to write about what skills they feel are important in leading groups with children. In their own words, here is what they said.

> Cutting off is extremely important. The kids are constantly interrupting each other. I find that if I go around the circle and give each a turn to talk and then go back around again, this works better for many of the younger kids, instead of skipping around like you would with adults. Kids respond a little more to the structure of knowing when it is their turn to talk. Also, keep tables away from kids. Definitely work with them in a circle with no table in the middle. They are more open with each other this way, tend to get less distracted, and tend to look down less. Keep extra chairs in the group. Kids like to slide over to an empty chair next to them. For ADHD kids, this gives them movement opportunities without being

disruptive. Have books, pencils, and paper on the floor—not on their laps—because they fiddle and doodle too much. Having them repeat what someone said in their own summarizing way and having them give advice to each other are good techniques. Also, have one student tell his or her story and then ask another student how they would feel if that had happened to them. In other words, they talk for each other on the tough stuff. In schools, allow 20–30 minutes for a group session. Less is not enough, and you lose their focus more easily with much more than 30 minutes. They like movement exercises. They love to act out things, so drama works well for kids in groups.

*Shelly*

In children's groups, you absolutely are going to lead 95% of the time if you want kids to get anything out of the group (and I am speaking elementary levels), because developmentally, they can easily get off the subject. Movement exercises are very important. More teaching tends to take place than with adolescents or adults. I use smaller member numbers than in adult groups. Six is definitely enough in elementary groups even when the kids can focus individually for longer periods of time—four members in ADD groups.

*Chris*

Kids are often thinking really hard about what they want to say when they are called on to talk. They want to contribute to anything that is talked about, but then they forget to listen carefully to what others are saying. I am constantly trying to make connections between group members to keep them focused and to help them learn something from the other members. For example, I often comment that two members are going through a similar situation, or ask for members to make a comment or suggestions about what someone else said.

Sometimes kids come in and aren't ready to do work, so I may give them 1 or 2 minutes to talk about whatever they want before opening the session. Then they have to be ready to talk about real issues. This strategy is incredibly effective! Kids have a shorter attention span, so you have to change the activities and format more frequently from talk and games, to books, and so on.

Behavioral strategies also work really well. For example, every 5 minutes, everyone who is on task gets a sticker. At the end of the 40-minute session, everyone who has six stickers gets a prize. This is better for grades K–2. Unfortunately, kids sometimes have to be extrinsically motivated to pay attention. Hopefully, the topic and activities are relevant and captivating, but it doesn't always work that way.

Give them things they can take back to class or home, such as pictures or a card with things about themselves written on it, and so on. This provides a means for them to share with their parents and reinforce what was discussed in the group.

*Dana*

I have recently been working in groups with children ages 4–6, and I have found that an understanding of holding, shifting, and deepening the focus, a clear purpose, and being creatively flexible have been the keys to successful sessions. Children at these young ages, they're not developed enough yet to express themselves through words. They speak through their actions and interactions. Lots of time is spent in play.

A clear purpose is imperative when leading groups with children. Children will mentally float, daydream, and get excited—that is part of the experience of being a child. My job is to point and guide these free spirits towards a common goal. When I work with children I am always prepared to validate their contributions and use them to further the purpose. For example, I was working with a group of six four- to six-year-old children, all in foster care. The purpose of the group was to normalize their experience in foster care and break their emotional isolation. We were working on an exercise to identify the job description of a parent. As we were listing the "dos" and "don'ts," one 5-year-old boy picked up a doll and began banging the doll off of the table. With a big grin the boy stated, "The daddy doll is making noise." Using this boy's expression and focusing the contribution to the purpose, I used a round to ask the other children "whose parents make noise?" The children giggled, and then each explained how their parents act around them. This assisted the children with accepting that other children's parents behave similarly to theirs, and provided the opportunity to release emotional discomfort. Without understanding how to manage the focus, clarity of purpose, and being creatively flexible, a young children's group probably will be a struggle.

*Earl*



**GROUP COUNSELING SKILLS: Counseling group on divorce**

Watch again video 17.4—a counseling group for children of divorce. This shows the leader working on the topic of self-blame. The leader uses a variety of skills, techniques, and theories.

# Adolescents

Adolescence (ages 12–19) can be a difficult period in a young person's life. Groups can help with identity problems, sexual concerns, and problems with friends, parents, and school. Groups for pregnant teenagers, drug users, teenage parents, potential school dropouts, and runaways can be extremely helpful. Other valuable groups are those for adolescents who are having problems because of their parents' divorce, remarriage, or alcohol abuse. Many experts have said that teens listen more to their peers than to adults, so groups can serve as a great source of learning and exploring. There are many activity books on working with different adolescent groups.

## Kinds of Groups

Groups for adolescents are both voluntary and nonvoluntary. The voluntary groups usually take place in schools, churches, or mental health settings, whereas the nonvoluntary groups are conducted primarily in treatment centers, residential settings, or mental health centers. Voluntary growth, discussion, education, counseling, and therapy groups can be quite valuable during adolescence.

The following is a list of many groups that may be found in a middle or high school setting. Most of these groups would be voluntary, although some may be mandatory.

Self-esteem or self-concept
Assertiveness
Anger management
Study skills
Drug education
Drug abuse
Post high school planning
Teenage fatherhood
Dropout prevention
Improving academics
Friendship
Divorce and stepfamily issues
Grief and loss
Sex education
Dating
Gay and lesbian issues
Teenage pregnancy
Teenage motherhood
Living with an addicted person

The following are groups that would take place at a mental health center, treatment center, or detention center.

Drug abuse
Survivors of sexual abuse
Living with an addicted person
Pregnancy
Teenage motherhood
Teenage fatherhood

Probation or parole
Gay and lesbian issues
Self-esteem building
Cooperative living (for residential treatment centers and group homes)

## Screening

For school groups, it is important to screen members to make sure they are appropriate for the group that you are forming. The best way to screen is to conduct a brief interview and find out why the potential member wants to be in the group. Young students often will sign up for group just to get out of class or to be with their friends. For a more in-depth discussion of the various aspects of recruitment and screening of minors for group counseling in both schools and agencies, see Ritchie and Huss (2000).

Depending on the group's purpose, the leader may want to lead all males, all females, or a mix of the two. Because there is a lot to learn about the opposite sex during the adolescent years, a coed group can be beneficial. The disadvantage of mixed groups is that members may be inhibited when members of the opposite sex are present.

In an ideal situation, the leader always has a chance to screen members, but this is not how things are in many counseling settings. Any discussion of adolescent groups would be remiss if it did not touch on the nonvolunteer adolescent group that the leader does not get to screen. These groups may be school ordered, court ordered, parent ordered, or agency ordered. Many schools have mandatory groups for those caught with drugs, those with too many absences, those with poor grades, or those who want to quit school. Courts sometimes order teenagers to participate in group therapy with an agency. Many residential settings have mandatory group attendance.

## Group Size

For best results, there should be no more than eight members in growth, support, counseling, or therapy groups, with 6 being ideal. If the group is more psychoeducational, then as many as 12 members may be appropriate, although it is usually best to keep the number to 10 or fewer.

## Length of Session

Sessions with adolescents should last between 40 and 90 minutes. Groups in schools usually last one period, which is around 40 to 50 minutes. In residential settings, sessions may last longer, especially if some time is spent on "house business." In mental health centers, groups usually meet at least 90 minutes.

## Number of Sessions

The number of sessions depends on the kind of group and the members. Some will meet for only one or two times. Others, such as drug and alcohol groups,

groups discussing gay and lesbian issues, and probation groups may meet for 10 weeks or even an entire year.

## Special Skills

A group leader working with adolescents should like and respect teenagers, want to learn more about their immediate world, and understand the kinds of struggles they go through while trying to grow up. Teenagers are very aware of phony attempts by the leader to be one of them or of a one-up attitude that suggests "the adult knows more." Adolescents will often test the leader's level of acceptance of their values. Unfortunately, it is common for group leaders of teens to sound like parents. Students can quickly spot hidden agendas, such as wanting the members to study harder in school, stop using drugs or drinking, or behave better in class. This can cause the members to lose respect for the leader. Leaders of adolescent groups are often confronted if they are dishonest or not open about their intentions.

Three important skills are (1) take charge, (2) use structure, and (3) make it interesting.

1. ***Take Charge***. It is very important with teenagers for the leader to establish that he is in charge and that he will lead the group. If the leader does not take charge, one or two of the members will, and the group will most likely go in a nonproductive direction. If the leader takes charge and makes the group interesting, the adolescents will not mind. Remember, *people don't mind being led when they are led well.*
2. ***Use Structure***. Because adolescents often do not come prepared to discuss issues, planning activities or topics is usually the best way to structure the group. Leaving it up to adolescent members to come up with things most often leads to irrelevant and shallow discussions.
3. ***Make It Interesting***. Any group leader who does not make her group interesting will be asking for trouble, because teenagers are used to the Internet, video games, TV, and so on. It is the leader's responsibility to make the group interesting by introducing stimulating topics, using relevant and interesting activities, and using a multisensory approach that keeps the members engaged. This is especially true for nonvolunteer groups, because they are extremely difficult to lead. The leader will have to be creative and innovative to turn the negative energy around. With adolescents, especially in the initial sessions, the leader must plan interesting and relevant activities. The use of role-plays, moral dilemma exercises, sentence completions, and common readings is helpful for getting members interested and involved. Starting with a formal presentation of the rules is *not* the way to start a nonvolunteer group—this sets a negative tone for members who are already negative.

During the first couple of sessions of a nonvolunteer group, the leader should expect negative behavior, both verbal and nonverbal. Sometimes it is a good strategy to allow members time to complain during the initial session.

One counselor we know started her after-school group for adolescent drug users by dealing with their anger at being in the group.

**LEADER:** I know you do not want to be here, so we can take 10 minutes to complain about it, but after that we'll get focused on what we are doing here. If you want to complain, I am going to put this trash can in the center of the group. In 10 minutes, after you have dumped much of your anger, I'll remove it and we'll get started.

Problems with confidentiality can often occur in groups with adolescents. Sometimes the leader can avert these problems by carefully screening the members to avoid placing members who do not get along in the same group. Because confidentiality is so important, the leader must make clear to the members the consequences of breaking that confidence. Leaders should also make members aware that confidentiality cannot be guaranteed. The consequences of breaking confidentiality may be discussed and agreed upon by the entire group, or the leader may simply state the consequences, such as removal from the group. The leader also needs to inform the members that she may have to tell parents about certain things that are said, if she feels the person is in danger of hurting himself or is engaging in behaviors that are very harmful or self-defeating. Agency or school policy, plus state laws, may dictate what a leader can and cannot keep confidential.

### Comments from a Practicing Counselor

Adolescents are an exciting breed. Their search for "self" and desire to belong can make group counseling quite a challenge. Most importantly, in my view, is the strong need to build a trusting relationship within the group. Activities that build rapport and trust are an absolute. Confidentiality issues also appear to be of extreme importance to this age group. It is important to offer private counseling for your members when you recognize that certain issues may best be addressed outside the group.

Cutting-off skills are crucial. As the group's leader, it is imperative to possess skills to be able to halt the domineering adolescent member. I've seen this type of child begin to exaggerate or fabricate stories and situations once they've realized they have the members' attention. To protect this member's acceptance by the others, it is important to recognize this and come to their aid by using effective cutting-off tactics. Effective planning is an absolute must for leading adolescent groups.

*Wendy*

## Couples

The value of couples groups lies in hearing what other couples are dealing with and doing about various issues in their relationships. Hearing others helps to stimulate thought and discussion in the group because members bring up

different ways of handling problems and topics that couples have either avoided or of which they have not been consciously aware.

Couples' groups are usually conducted at a mental health center, treatment center, or church. The church groups are mostly enrichment-type groups, whereas the other settings focus on enrichment, support, or therapy groups.

## Kinds of Groups

Below is a list of the kinds of groups that are frequently offered to couples. Note that, as stated above, the kinds of groups can be dictated by the setting in which the leader is working.

***Enrichment and Support Groups:***

Communications
Living with differences
Living with religious differences
Living with racial differences
Dual-career marriages
Surviving the loss of a child
Living with a parent with Alzheimer's disease
Living with a chemically dependent child
Living with a mentally ill parent, child, or other person
Living in a prejudiced world
Living as a gay or lesbian couple

***Therapy Groups:***

Abused spouses
Healing from an affair
Healing childhood wounds
Stage II Relationships—beginning a new relationship after recovery
Weekend intensive work

## Screening

Because the needs of couples wishing to participate in a group experience can vary considerably, it is important for the leader to be clear about the purpose of a group. Through screening interviews, the leader can inform couples about the purpose and assess their needs. By meeting with the couple and each partner individually, the leader may get a broader perspective on the relationship. If one of the partners is not committed to working on the relationship, the leader may suggest couple counseling before or in addition to the group experience. Often, during screening, it will be evident that one of the partners needs some intensive therapy rather than a couples' group experience.

## Group Size

The size of a couples' group varies, depending on the format. Some are conducted in large groups of 10 to 12 couples. For the more personal groups, usually no more than 5 couples is best to allow enough time for each person to share his or her thoughts and feelings.

## Length and Number of Sessions

The format for couples' groups can vary. Many enrichment groups are held on an extended basis, for a full day or a weekend, the idea being that this allows time for partners to more fully explore the dimensions of their relationships and to work more intensely on issues (Corey, Corey & Corey, 2014). Full-day or weekend sessions also allow more sharing with and learning from other couples. A drawback to this format is that couples may leave the session with a group "high," feeling renewed and excited about their relationship, only to reenter the real world of kids, work, traffic, money, and in-laws. Their enthusiasm and renewed commitment may carry them for days or weeks, but long-standing, underlying problems can surface again for some couples under the pressure of everyday living. Many leaders who conduct such marathon couples' groups hold follow-up meetings to support the couples' changes and to help strengthen areas they have worked on.

A weekly or biweekly group session format can also be used with couples. However, couples—especially if they are parents—may have a problem finding time to attend a 2- or 3-hour group session on a regular basis for several weeks or months. If couples are willing to make this commitment, much can be done to help them look at their relationship. Couples can work on improving intimacy, handling disagreements, decision making, and so forth. They also have the opportunity to discuss and handle daily issues that might arise for them, such as parenting, money, personal time scheduling, sex, in-laws, and vacations. Weekly or every other week sessions allow couples to try out communication and problem-solving techniques over an extended period of time; if they run into snags, the group is there to help work them out. This format takes into consideration that personal change, especially when two people are involved, is often slow and erratic, requiring continued reinforcement to be lasting.

## Special Skills

Groups for couples offer some special challenges to the group leader. If at all possible, couples' groups should be co-led, and often are led by leaders who are partners themselves. The leader is not only dealing with the dynamics among six or eight members but also with three or four relationships that have their own dynamics. Couples' groups, therefore, are very complex and are usually difficult to lead. We briefly discuss some of the special skills for leading couples' groups.

1. *Understanding of the kinds of problems that may arise.* In a therapy group for couples, members may have different agendas. One member of a couple

may be coming to the group hoping that the relationship can be improved or saved, whereas the other is perfectly satisfied with the status quo or wishes to negotiate a divorce. Groups of this nature can become quite intense, because the two members often air their differences in the group. Sometimes this is very painful for the couple as well as for other group members who are watching.

Other problems that may arise during the group itself include the following:

- One or both partners may be reluctant to share personal feelings in front of the other.
- One partner may feel the need to conceal an affair.
- Couples may use the group to vent powerful negative feelings they have held in for a long time.
- One or both partners may try to use the group as a jury to justify their behavior in some way.
- One member in the relationship may try to enlist the group to change the partner's behavior.
- Members may compare themselves or their partners with others in the group and feel bad because they do not compare favorably to the others.

2. *Knowledge of the counseling theories.* Counseling theories are important to teach because so often couples are looking for information about why they do what they do to each other. Along with other theories, we always use transactional analysis (TA) because TA is excellent for understanding and explaining transactions between couples.
3. *Knowledge of many different kinds of couples' exercises.* It is useful to have activities for the couples to share with each other as well as with the group. Such exercises are helpful because they encourage couples to look at the broader issues that many people in relationships share, rather than to focus only on their own specific disagreements. Exercises help couples examine such areas as sex-role problems in the relationship, dependency, the need for innovation to keep relationships interesting, the need for separate identities, games that interfere with intimacy, and the role of childhood experiences in shaping values about relationships and marriage. Hearing other couples discuss their reactions to the exercises is very valuable for the members. Excellent exercises can be found in various sources available for leaders working with couples' groups (Corey, Corey & Corey, 2014; Jacobs & Schimmel, 2013; Stevens, 1972).
4. *Use of drama and psychodrama.* In many couples' groups, drama and psychodrama are used to get at the deeper emotions that exist. By seeing other couples work on issues and situations, other members find that they can relate to scenes being acted out.

5. *Working with couples who have special needs.* For gay and lesbian couples' groups, the leader must know and understand the particular issues facing these couples. This knowledge comes from reading and from interacting with those living this lifestyle. It cannot come from one's own imagined ideas as to what this is like, and it is unethical to lead these groups without knowledge. Often someone who is lesbian or gay leads these groups. Other kinds of couples' groups include parenting groups and groups for parents with children who are addicted or who have special needs. Much literature can be found on leading these groups.

In summary, couples' groups are exciting, dynamic, and challenging. Couples' groups call for an in-depth understanding of relationships as well as the ability to handle complex dynamics that often arise. Group leaders working with couples need to have strong leadership skills and extensive experience in individual and couples' counseling.

### Comments from a Practicing Counselor

It is so important to be clear as to the purpose of any couples' group. Marriage education is about teaching skills to couples, whereas therapy groups deal with specific issues of couples. Screening is key for all couples' groups. In my therapy couples' groups, I made sure the partners were functioning well enough to be in a group setting rather than just couple counseling. You have to cut off and redirect a lot because partners try to blame, shame, and focus attention on their partner. If a member talks about his or her partner, I throw it back on to the person doing the blaming. This tends to stop attempts to put the partner in the hot seat. Facilitating ownership is an important skill.

*Pat*

## Addicted Clients

Many different groups exist for people who are addicted to some substance or activity. We took the liberty of putting many different and unique populations into one category, but we feel that in this condensed discussion, the reader can get a sense of the challenges of working with anyone who has an addiction. The groups discussed in this section could apply to any addiction, be it overeating or not eating, shopping, gambling, sex, or drugs and alcohol.

Many groups are available to the addicted individual. Twelve-step support groups that are usually not led by a professional—such as Alcoholics Anonymous (AA), Narcotics Anonymous, Overeaters Anonymous, Sex Addicts Anonymous, and Gamblers Anonymous—have benefited millions of people. For each of these populations, education groups led by a trained therapist can provide valuable information. Also, therapy groups are beneficial for those who are in denial or are

still struggling with their addiction and its consequences. A search of the Internet can lead you to Web sites for the topic in which you are specifically interested. These Web sites contain references to books and materials that can get you started with ideas for leading any kind of addictions group. Our discussion is only meant to give you some overall understanding of these groups, but you will definitely need much more information than is provided here.

The value of addictions groups cannot be overstated. Addicts need to hear from other addicts, especially those who are struggling and those who are doing better than they are. Much support and help can be given because members feel a strong bond with those who are living what they are living. Also, other addicts can call each other on their "stuff."

## Kinds of Groups

Any discussion of an addiction group has to be divided into two groups: nonvoluntary and voluntary.

**Nonvoluntary Groups** Nonvoluntary groups for addicts consist of those who were caught driving under the influence or those who were mandated to attend by their employer, school, parent, or some other outside authority. These groups may be strictly educational, or they may serve the purpose of breaking the addicted person's denial that nothing is wrong. These groups take place in mental health centers or treatment centers and should be led only by persons who have extensive knowledge of the addiction they are addressing.

**Voluntary Groups** There are many voluntary groups for those who have some addiction that greatly affects their lives. Those addicted who want help often volunteer for recovery, therapy, and relapse-prevention groups. Any such leader must be well versed in the specific addiction to lead these groups, because the power of the addiction is so strong that much manipulation, pain, and struggle will be present.

## Screening

Ideally screening is done for all groups, but with addiction groups this often is not possible, because the members are mandated to the group or are confined to a residential treatment center or prison that requires everyone to attend a group. For volunteer groups, leaders should screen for commitment to change. Too often, people come to appease a spouse, parent, or boss. These members can be trouble, especially if they try to sabotage the group process. For those groups where attendance is mandatory, some leaders use an inner circle/outer circle concept for each session. They have those who want to actively participate pull their chairs to the center, and those who just want to listen sit on the outside. Theoretically, this keeps those on the outside from offering distracting comments, and it allows other members a chance to go deeper with the session.

## Group Size

The ideal size is 6 to 10 members for a therapy group, depending on the members and the purpose of the group. Educational groups can be larger, and AA type groups can have 30 to 40 or even more members, because they are open to the public.

## Length of Session

Alcoholics Anonymous groups and other similar groups usually last 1 hour. Groups led by a therapist usually last 1½ to 2 hours but may be longer in some residential or treatment settings. In some treatment centers, groups meet daily, so the leader has to have a tremendous amount of knowledge and excellent skills to make it interesting and valuable every day.

## Special Skills

1. *Dealing with anger.* The initial sessions of any nonvoluntary group for addicts are difficult to lead because the members do not see themselves as having a problem. The leader must be prepared to deal with the members' anger at being forced to be in the group and the anger members feel when the leader refers to the fact that their addiction is creating problems in their lives.
2. *Use of a multisensory approach.* There are no easy answers to the problems of working with nonvoluntary addicts. Quite often the use of interesting movement, writing, or prop exercises can be very helpful in "hooking" the nonvoluntary member. Leaders who use a multisensory approach are almost always more successful than those who rely on just talking.



**GROUP COUNSELING SKILLS: Therapy in Groups**

Go to video 14.1 and watch the segment where the leader uses multisensory techniques (the hammer and the large beer bottle) when working with one member.

3. *Handling confrontation.* Besides having a thorough knowledge of addiction and group leadership, the leader has to have the courage to be confrontive and be comfortable with members confronting each other. The leader must always remember that effective confrontation involves the leader or another member confronting a member about inconsistencies. Too often, inexperienced leaders either attack the member or let other members viciously attack the person. This is usually not effective and can be quite harmful. Leaders can easily become frustrated with

members who deny their problems and may use confrontation in a punitive way—to attack, belittle, or humiliate the member. If a leader finds herself feeling anger toward a member, she should examine her own expectations for the member and perhaps talk with a colleague about her frustration. Constructive confrontation is valuable and often necessary. It involves statements such as, "You tell us you are going to get sober, yet you have no real plan for how you are going to do it. That's why none of us believes you." The use of confrontation requires skill and sensitivity.

4. *Planning.* Careful planning must be done for most groups of people with an addiction, especially in the beginning. For any nonvolunteer group, the leader should realize that the members are often defensive about their addiction and hesitant to talk about any of their issues. The leader has to plan relevant, interesting exercises and discussions to get members involved and committed to trying to benefit from the group.
5. *Cutting off.* One other skill that is absolutely essential when leading groups for addicts is cutting off. The leader needs to use cutting-off skills because addicts, especially alcoholics, often want to tell their story over and over again. Many leaders who have worked with alcoholics have said that cutting off the long-winded stories has proven very valuable in allowing for more sharing and interaction, and in focusing on a topic in depth without having to listen to one story after another.
6. *Patience.* Leaders must be patient and understand that they will not see quick and dramatic results. Always keep in mind that this is an extremely difficult population with which to work. The success rate is not as high as group leaders might find with members who have other types of problems. Group leaders in the addiction area should not expect rapid, large-scale behavioral change. Leaders should be especially careful to avoid burnout by having realistic expectations and not leading an excessive number of groups in a given day or week. Leaders want to make sure their psychological worth is not attached to clients getting better, because many addicts relapse or simply don't work any kind of recovery program. It is important not to feel personally unsuccessful as a leader or counselor when members return to the treatment program for the same problem.

## Comments from a Practicing Counselor

The need to be creative is extremely important. This is especially true with those clients mandated into treatment. I use a lot of movement exercises.

Cutting off and drawing out are essential tools when working with substance abuse clients. Oftentimes, members get stuck in "telling their story." Although this can be very beneficial, I have found that I need to be attuned to their tone of voice and their purpose for sharing. Other

members are reluctant to share and will try to hide in the group. This is especially true for women, specifically when the group is made up of considerably more men than women. Breaking members into dyads or triads has proven to be very effective. Also, written exercises often focus members. I have done an exercise prompting clients to reflect and uncover their ritualistic behaviors as they relate to their alcohol or drug use. This has been very helpful, not only in breaking denial, but also in having members identify with one another.

*Trish*

## Older Clients

As with other special populations discussed so far, there are some special considerations to be aware of when working with groups of older persons. Although we speak of "older persons" or "the elderly," these labels cannot possibly accurately describe those over a certain age, because many people in their 80s function as well as those in their 50s. As group leaders, we must be aware that the needs of the elderly are similar to the needs of us all; that is, their needs are very diverse. Older persons may feel they have less personal power and impact than they had previously in their lives when they were employed, had children who sought their advice, or had friends who relied on them. This may lead to feelings of lowered self-esteem or alienation. Older people may have less contact with family members, siblings, or children than in previous years. They may also have fewer social contacts, their friends having died or moved away. This can lead to feelings of isolation and depression. Older people, having realistic concerns about physical and mental infirmity and death, can benefit greatly by being in a group where they can talk about these feelings. The group leader must be comfortable with dealing with thoughts and feelings about death.

Older people are often preoccupied with past events. They tend to reminisce and tell stories from the past. Although such storytelling may become boring for other members, it is often a healthy way for people to process their lives and feel good about themselves. A very common activity in groups with older clients is reminiscing about different aspects of their lives. For example, the leader might allow a certain number of minutes for each member to recall his most memorable holiday, a particularly happy moment with a spouse or friend, or the most rewarding experience she can recall. These exercises can elicit memories that might lead to the member's wanting to do some personal work. These groups for older adults also serve the purpose of fulfilling social needs that they have but have few outlets for meeting these needs.

### Goals and Kinds of Groups for Older Persons

Although older persons can benefit from personal growth and counseling groups, the leader may find as much interest and response among older members for task, support, and education groups.

Following are five possible goals of groups involving this population.

1. *Providing information.* The first goal involves providing information about various matters such as finances, health and medicine, housing, or insurance. The leader can either present the information or arrange for guest speakers.

2. *Making things happen.* Another goal is what we call "making things happen." Older people, especially if they are living in a residential facility, often do not have the resources or the influence individually to make things happen, such as changing a schedule, planning a holiday party, improving living conditions, and so forth. A task group may take on such goals to improve the general quality of life. Such a group can also provide an opportunity for members to learn or relearn effective assertiveness skills.

3. *Working through personal issues.* A third goal involves working through personal issues. A personal growth or counseling group can help members deal with unfinished business from the past, such as guilt about not being a better parent or spouse, anger at a former friend or employer, or sadness at not having accomplished something during their lifetimes. Issues such as their treatment by children or other family members or their concerns about the future, infirmity, or death are also potential areas for personal work. Members also benefit simply from sharing these concerns and finding support within the group.

4. *Developing and maintaining social contacts.* The fourth goal for groups with older persons is developing and maintaining social contacts. As people grow older, family contacts may be less frequent because of distance or their family's preoccupation with their own lives. Friends move away or die, neighbors change, longtime neighborhood hangouts close, and the opportunity for continued social contact seems to diminish. Making new friends can seem difficult, and therefore some older people tend to become more introverted and preoccupied with themselves and the past. This can lead to diminished social skills and diminished initiating behavior. A support group can provide a place for members to socialize—to regain former social skills—as well as to meet others with whom they may form more lasting relationships.

5. *Explore new goals in life.* Finally, we have found that groups can be very useful in helping older persons explore new goals in life. Such a group may have characteristics of an education group, a support group, and possibly even a growth group, as members shift their preoccupation from the past to focus more on the present moment and the future. The leader and other members may provide information and ideas for specific projects, such as arts and crafts work or community activities such as joining a foster grandparent program. New goals may involve

writing to old friends, establishing closer ties with other family members, or pursuing goals formerly abandoned, such as writing or taking college courses. Members can learn that, whatever their medical or health restrictions, it is possible to generate a new and continuing interest in life.

## Settings

There are four primary settings where groups for older persons might be held: (1) the community, (2) minimum-care residences or retirement facilities, (3) nursing homes, and (4) institutions. We briefly discuss the implications for each of these settings.

*The Community* There are older persons in most communities who do not have contact with family or friends and who may live rather isolated lives. Some may be in touch with local social agencies. A group program may be developed for such persons to pursue any or all the goals discussed before. Such groups may be sponsored by the community mental health center, schools or colleges, local churches, or senior citizen centers.

*Minimum-Care Residences* Standard living facilities designed for older persons also provide a setting in which such groups might be held. Although such facilities may have professional staff qualified to lead groups, it is more likely that a group leader from the community will provide this service.

*Nursing Homes* Nursing homes need some kind of group program. The severity of a person's physical illness obviously places some constraints on involvement; however, the need may be just as great. Members should be screened for both their physical ability to participate and their mental status.

*Institutions* There is a great need for groups for older persons in institutional settings. Many of the goals discussed here can be pursued effectively in such a setting. Institutional living can be very difficult and, coupled with medical and psychiatric problems, can lead patients to become depressed and socially isolated. A group program can go a long way toward helping with resocialization, developing a support network, and providing information and guidance. Corey, Corey, and Corey (2014) provide a warm description of the contributions Marianne Corey made to a group of older residents in an institutional setting.

## Screening

Screening groups for the elderly is very important and will depend on the purpose. If the purpose is to teach something new or have members focus on some task or topic, then it would be important to select members who have the ability to learn and focus. For groups where focusing on memories

of the past is a primary component, the leader will want to make sure that selected members are able to access their memories, which is a difficult task for certain older people.

### Group Size

The size of the group for the older population depends on the kind of group and its purpose. If it is more for social interaction or education, 10 to 12 members is fine. If the group is more for support and sharing, 4 to 6 members usually works best.

### Length of Sessions

Sessions usually should last between 1 and 1½ hours, depending on the level of functioning of the members, the group size, and the purpose of the group.

### Special Skills

The following three skills are just a few that leaders should possess in group counseling for older clients.

1. *Patience.* Probably the most important skill in working with the older population is patience. Things tend to move at a slower pace, and the counselor needs to understand and adapt to this.
2. *Energy.* A skilled leader will have lots of positive energy in hopes of energizing those who are not so enthusiastic. An upbeat, positive attitude will usually help the group go more smoothly.
3. *Knowledge of certain issues.* The leader working with older persons must be sensitive to relevant issues, such as concerns with aging and death, as well as to real constraints, such as waning physical health and vitality, and fewer social outlets. The leader should develop a group program that will meet a wide range of member needs.

## Clients with Chronic Diseases or Disabilities

Groups for persons with chronic diseases or disabilities can be formed on a community basis or in hospitals or rehabilitation centers. Such groups are especially important in light of the stress generated by the onset of a disability or long-term or terminal illness. Clients are often uncertain of the course or prognosis of their condition. They must deal with a change in lifestyle, the feelings of loss of control, and the physical separation from family and friends because of being in a medical setting, perhaps in a different city. The focus of their lives for many weeks or months shifts from the day-to-day business of

making a living or attending school, socializing, and being involved with family and recreation to medical examinations, treatments, and waiting with some degree of uncertainty.

## Kinds of Groups

Given the uncertainty of medical factors, the need for emotional support, and the struggle of working through lifestyle changes, different kinds of groups with this population can be quite beneficial. Groups can provide education and information on health-related issues and help clients deal with psychological issues such as loss of identity, anger, and the grieving process. Groups can also provide support and help with problem solving. Many excellent support groups are being led for all kinds of medically related issues. If you are planning to lead these kinds of groups, you should be aware that much literature discussing different approaches to each is available online and in new books that are out.

## Screening

When possible, it is usually best to form homogeneous groups based on one particular illness or disability, because the illness or disability is the common denominator about which patients have concerns. There will be times, however, when this is not possible or when it is helpful to involve group members with different medical concerns. The leader must give a great deal of thought to who is let into the group and must be especially sensitive to the intense anxiety, depression, and anger that people may experience following the onset of a long-term illness or disabling condition. It is important for the group leader to understand the grieving process that people go through when faced with a long-term illness or disabling condition, both in terms of selecting members for a group and in terms of actually leading the group. Members who are dealing effectively with their medical condition can provide information, lend support, and serve as role models for those who are still in the angry or depressed stages.

Groups with members who have a chronic disease or disability are led both as open and as closed groups. Most hospital groups are open because of the need to accommodate new people arriving in the unit who are suffering from the same illness or disability. Groups for family members of people with certain illnesses are also being conducted with great success.

## Group Size and Length of Sessions

The size of these groups varies according to their purpose. Educational groups can be large. Support and therapy groups should probably have no more than eight members, with six often being an ideal number. These groups usually meet for 1½ to 2 hours because members usually have many thoughts and feelings to share.

## Special Skills

1. *Dealing with powerful feelings.* Leaders of groups that deal with a disease or disability need to be prepared for many different dynamics. Because of the sense of injustice members feel at the onset of a chronic or terminal illness or disability, they often have a strong need to express powerful feelings—frustration and anger, in particular. For this reason, group leaders must feel comfortable handling hostility, some of which will be displaced onto them. The anger often takes the form of finding fault with the medical staff, nurses, schedules, and so forth. It is important for the group leader to recognize this anger as an important aspect of the grieving process and to help the member work through it.
2. *Getting members to focus on other aspects of life.* Members often become intensely focused on their medical conditions to the exclusion of other aspects of their lives. They may need to be encouraged to look beyond their medical condition to the possibility of future employment, socializing again with friends, regaining a role in the family, and so forth. It is important to use effective cutting-off skills along with exercises that structure how the members share. For example, sentence-completion exercises are helpful, because the leader can focus members on certain aspects of their lives and illness by the sentences he uses.
3. *Letting members be experts.* Leaders, especially beginners, need to understand that clients with a chronic disease or disability often become quite knowledgeable about their condition. Rather than be threatened by the members' knowledge, leaders should try to learn from them.
4. *Cutting off.* Certain members may try to use their superior knowledge about their condition to manipulate the leader, attempting to dominate the discussion or discount the leader's effectiveness because of lesser knowledge. The leader may need to use cutting-off skills to make sure that such members do not sabotage the purpose of the group.
5. *Being knowledgeable.* Having as much knowledge as possible about the disease or disability is always helpful. A few illustrations may help to emphasize this importance. For example, when working with young men with spinal cord injuries, sexual dysfunction is frequently a major topic that members wish to discuss. Often there is a high degree of misinformation about sexual problems that circulates among such patients. The leader will need to have—or make available through medical staff—sound information on the disability and sexual dysfunction. Also, he needs to anticipate the anxiety surrounding this topic. Group members who have had a stroke may experience mood swings caused by their physical condition, depending on the nature and extent of brain damage. It is important for the leader to be aware of the cause of such mood swings, to make sure the affected member knows the cause, and to ensure the other group members understand. Stroke patients may also experience difficulty expressing themselves at times or fully

comprehending others' ideas. Much support and encouragement must be given to such members to keep them involved in the group. Cardiac patients may develop a preoccupation with death and a concern about how much physical energy they can expend safely. Again, misinformation is common; even accurate information is often not fully comprehended because of these patients' high anxiety levels. Education, along with support and encouragement, is important when working with cardiac patient groups.

6. *Understanding the psychological ramifications of being in a residential treatment center.* A final consideration in working with group members with problems of chronic disease or disability in a residential treatment program is the effect the setting may have on members' outlook and behavior. Most hospitals and rehabilitation facilities regiment behavior and foster dependency. Patients are frequently told when to get up in the morning, when to eat, what activities to attend, when, if, and how often they may use a telephone, and so forth. Patients may even develop a belief that their lives are literally in the hands of the medical staff. Regimented thinking and dependency may lead to apathy on the part of such group members. Leaders may find the need to work with both staff and group members in countering this apathy.

## Survivors of Sexual Abuse

Sexual abuse of children and adolescents is a significant mental health problem. To help people heal from the abuse, groups for survivors are being conducted for teenagers and adults in such places as mental health centers, abuse shelters, and drug and alcohol centers.

### Value and Kinds of Groups

Survivors of sexual abuse are likely to benefit from one of two kinds of groups: support or therapy. Most of these groups are therapy groups because the content relates to painful events and memories. Groups provide an opportunity for survivors to speak of their pain, perhaps for the first time, with others who have also been victimized as children or adolescents. These discussions can be empowering because they validate the abusive experiences and cut through the veil of secrecy that most survivors were subjected to as children. The chance to speak openly in a supportive, confidential setting often helps survivors gain courage to work more productively in their individual therapy.

### Screening

When establishing a group dealing with abuse, the leader must consider the maturity level of potential members. We feel that the minimum age should be 12,

although in most cases, 13 or 14 is a better age. We usually recommend that a potential member become involved in individual counseling before joining a survivors' therapy group. Individual therapy gives survivors a chance to gain an initial understanding of what happened to them. It is not unusual for survivors to deny or rationalize their abuse or to feel overwhelmed with shame. Survivors can benefit more from a therapy group when they have passed beyond their denial and are firmly committed to healing from their abusive experiences.

An individual interview with the leader before the first group session is especially important in a survivor's group. Members should be informed of the purpose of the group, its goals, the frequency and length of sessions, the group rules, and the techniques that will be used to help members work therapeutically on their issues. The interview should serve as a time for potential members to decide whether they feel ready to be in a therapy group. A member who is not ready for open discussions or is ambivalent about being in the group may feel too vulnerable to participate and may want to continue with individual therapy.

## Size of Group

Most of the time, these groups are quite small. We recommend no more than six members and often as few as three or four, depending on the members, their ages, and their stage of healing.

## Length of Sessions

Sessions should last 2 to 2½ hours to allow time for members to go to a deeper level and have ample time to come out of the pain. Sessions shorter than 2 hours may leave members feeling "unzipped."

## Number of Sessions

Survivor work in groups may take many sessions. However, because of the pain and distress survivors may experience, it is important to keep members motivated to work on issues. We suggest setting up groups on a time-limited basis—6 or 8 weeks. Although the group will likely continue for many months, these benchmarks allow members to assess their progress and decide whether they wish to continue or take a break. It is especially important to avoid leading an ongoing group where the members and the leader become comfortable with meeting but are not doing the work that is needed.

## Special Skills

Leaders of survivors' groups should have considerable knowledge of the issues surrounding incest, sexual abuse, and trauma. We often encourage adults who were abused to read the book *Healing from Trauma* by Cori. Classes, readings, workshops, and ideally leading under the supervision of a more experienced

leader should be aspects of the leader's preparation. If the leader is also a survivor of sexual abuse, she should consider the progress she has made in her own healing. As is true in leading any group, personal issues that the leader has yet to resolve could interfere with the leader's role or effectiveness.

Therapy groups for survivors focus in-depth on the pain of the individual members. However, this does not mean that the members must discuss details of their experiences in the group setting. The extent of detail discussed will depend on the comfort level of the members and the therapeutic needs of each member.

Because of the potentially intense reaction of individual members when exploring their abuse, we recommend that the group leader work with one member at a time rather than, as might be true in other kinds of therapy groups, attempting to take the entire group to a deeper level. Even the most skilled leader will be ineffective in trying to deal with the severe pain of four or five group members simultaneously. Leaders will soon discover that most members of survivors' groups can benefit greatly from watching other members explore their issues.

We support the idea that those in a survivor's therapy group either remain in individual therapy or, at least, maintain periodic contact with their therapist. In many instances, the individual therapist will be the group leader. It is important for members to have a "safe person" with whom to process their experiences if the group experience becomes overwhelming.

## Divorce Groups

Many mental health centers and churches offer groups for those going through a divorce. Groups for those divorcing fall into two major categories—recovery groups and starting-over groups. Recovery groups help people feel that they are not alone in their pain over their divorce. Hearing and helping others with similar issues can be quite therapeutic, and groups can help the members not focus so much on themselves. Also, sometimes the pain is such that a member may feel there is no hope until she sees other members progressing through the stages of grief. Recovery groups are usually short-term groups, lasting 6 to 8 weeks. Recovery groups can also be either open or closed. Ideally, they would be closed, but because clients at an agency may need such a group, having the group be open might work best for the population being served. Many people who complete a recovery group go on to a starting-over group.

Starting-over groups help members to do just that—start over. Support, sharing, caring, and risk taking can be experienced in a starting-over group. Often feeling a sense of belonging is quite helpful for those who do not have a local support system, because family and friends are either far away or nonexistent. If they are closed, starting-over groups can last for several months, or they can be open and last indefinitely.

A third category of divorce groups is the parenting group, intended to help divorcing parents with single parenting and the shared parenting process. Groups for single parents and non-custodial parents are being conducted in churches, mental health centers, and schools.

## Screening

When setting up a divorce group, the leader must be clear about the group's type and purpose. If the leader is not clear, then the composition of members may be such that the group cannot be successful. Divorce recovery groups consist of members who are currently going through a divorce or are recently divorced and are still very much grieving the loss. For these groups, the leader wants to make sure the member is not in so much pain that he would not be ready for the group. Some people need time with an individual counselor before joining a recovery group. Starting-over groups are for people who have worked through the initial pain and loss on their own, with a therapist, or in a group, but who are still looking for help and support in fully letting go and getting on with their new life.

## Group Size and Length of Sessions

These groups should not have more than eight members, because there must be time for each person to share. Often these groups end up having only three or four members, which is fine, because the members are usually willing to talk about their issues. Sessions usually last between 1½ and 2 hours.

## Special Skills

A leader of a recovery group must have a thorough knowledge of grief counseling and not be afraid of the emotional pain that will be exhibited. Also, the leader must be skilled at individual counseling, because the leader often works with one member at a time. Working with one person at a time is usually quite effective if the leader uses theories that the other members can apply to their own situation.

Rational emotive behavior therapy (REBT) or cognitive behavioral therapy (CBT) is quite useful in recovery groups because most of the members are telling themselves a number of irrational sentences about approval, failure, and unhappiness. Many say to themselves that they don't think they can live without their partner or that they will never be happy again. Also, they may feel much anger and blame either themselves or their partners; this needs to be discussed and challenged. The skilled leader can use REBT or CBT to dispute these ideas. It is important for the leader to be strong in theory; she cannot rely heavily on the members for helpful input, because they all are telling themselves many of the same things.

The leader's role in a starting-over groups is to make sure that members discuss important issues, such as dealing with an ex-spouse, anger, issues with

children, meeting new people, and dating again. The leader has many options in the way this kind of group is conducted. She can let the members direct the focus of the group by letting them bring up topics that are relevant for them, or she can use a format where she brings up relevant topics at each session. Some leaders who prefer the latter model use one of the many books or workbooks that have been published about divorce. One excellent book we have found is *Rebuilding* by Bruce Fisher (2005).

Leaders of parenting groups need to have good knowledge of theoretical models of parenting such as Adlerian, TA, or behavioral. They should also have experience in dealing with the different issues that arise in parenting after a divorce, especially a divorce where the spouses do not get along and see the world quite differently.

## Adult Children of Alcoholics (ACOA)

Adults who grew up in alcoholic or addicted families often have problems stemming from all the chaos and craziness that occurred during childhood. Groups provide an excellent place for people to hear that they are not alone and that others have some of the same feelings, fears, and habits that they do.

### Kinds of Groups

There are three basic kinds of groups for adult children of alcoholics: leaderless support groups, leader-led support groups, and therapy groups. All these groups are valuable and have helped millions who have been involved with one or more of them. Our discussion will focus on leader-led support groups and therapy groups.

### Screening

Leaders of support groups for ACOAs want to make sure the members are ready to be in such a group. If screening is not conducted, leaders may find they have members who actually should be in individual therapy rather than a support group. A support group can come early in a person's recovery and can help the person become aware of the issues that he has, or it can come when a person is in a later stage of recovery.

Leaders definitely should screen for a therapy group to make sure the members are appropriate. The leader will want to make sure that any potential member is able to function in a group. In addition, potential members should not be in such great need that individual therapy would be more helpful than group therapy.

### Group Size and Length of Sessions

For leader-led ACOA support groups, 8–12 members is good because you mainly want members to share and support each other. ACOA therapy groups

should not have more than eight members and may be as small as four because members are often working on painful, emotional issues. We recommend that ACOA groups last at least 2 hours because usually most members have much to share.

### Number of Sessions

ACOA support groups are often open groups, so they last indefinitely. Closed therapy groups last anywhere from 8 to 15 sessions and can be extended if the leader feels more time would be beneficial.

### Special Skills

Any leader of ACOA groups must have extensive knowledge about alcoholism and its effects on family members. A leader definitely needs to understand issues such as trust, intimacy, shame, guilt, and abandonment that plague most ACOAs. The leader must be prepared to deal with much emotional pain and anger. The Gestalt technique of talking to an empty chair that represents the alcoholic or addict has proven very effective. Using TA seems to give members a better understanding of themselves now and when they were living with an alcoholic parent. Activities such as family sculpture or psychodrama are extremely valuable if led by a skilled leader who can deal with the emotional material generated.

If you plan to lead these groups, be sure to get the necessary skills and knowledge. Too often, we hear of mental health workers leading ACOA groups when they do not have the knowledge or the individual counseling skills essential for leading such groups.

## Multicultural Issues in Groups

In thinking about multicultural groups or diversity in groups, leaders need to be aware of some specific situations. In the second part of this section, we discuss multicultural groups that are formed for the purpose of exploring and understanding differences. First, we focus on what most often occurs regarding diverse populations.

Frequently, leaders need to be prepared to deal with differences such as race, religion, and sexual orientation. Members of different cultural and social backgrounds will likely have different perspectives and values, and the leader needs to be prepared to deal with these differences if the differences cause tension or confusion among the members. If tension arises among members *because* of cultural differences, often the main purpose of the group cannot be dealt with until these issues are resolved.

When differences are obviously present, the leader has to decide whether to focus on the differences that have arisen. The decision to point out cultural differences among members depends greatly upon the purpose of the group. If a

counselor is working with a group of volunteer students on the task of planning a student party for a favorite retiring teacher, and the members are from various cultural backgrounds, cultural differences may surface in terms of suggestions for the celebration. The leader would probably not focus too long on the cultural differences, because they most likely would not play a major role in the planning of the retirement party. However, in a school group focusing on friendships that includes diverse cultural points of view, the leader may want to point out cultural differences that influence interpersonal styles and facilitate an exchange of cultural perspectives.

## EXAMPLES

This group consists of new students, both girls and boys, in a middle school setting.

**LEADER:** Now that we've gotten acquainted and spent time last week talking about your first weeks in a new school, I thought we might talk about how you are doing making friends here. Any thoughts or comments on how you are doing with friendships?

**AMY:** It's really hard coming to a new school. My mom thinks a party for some of my classmates would help me make some friends. I don't think anyone would come.

**LEADER:** That's a good topic. We'll come back to that.

**PETE:** I have been mostly hanging out where the guys play basketball. The other day, I just grabbed the ball and tried a basket from the side. I missed, but right away they asked me to play. I got two new friends already.

**LEADER:** So Pete, you found that by being assertive, you were accepted. How about someone else?

**KAMEKO:** I could never do that. In my country, we were taught to be patient. Wait and friends will come to you.

**PETE:** But you could wait a long time. Sometimes you have to show you want to become a part of their group.

**LIAN:** I am like Kameko. My parents have taught me that I may offend people by being too pushy, so I also wait until someone seeks me out.

**LEADER:** It sure seems that for Lian and Kameko, the way you were raised may be playing a part in your not making friends. Maybe some of you can share your thoughts on how the American culture works in regards to making friends.

In this example cultural differences have definitely emerged. The leader decides to focus on these differences, because it may help Lian and Kameko.

In this example, the leader, a pastor trained in counseling, is leading a premarital values exploration group for his parishioners in a large urban setting. There are five couples from various cultural backgrounds. The leader has spent time privately with each couple discussing their plans for marriage and what they might wish to gain from participating in six sessions of the group. It is 10 minutes into the second session.

**LEADER:** Joe, you seem like you have something to say.

**JOE:** Sally and I talked about how important it is for us to figure out how to have kids at an early age and still have both of us working. I don't think that's going to be easy. We both have careers to build, yet we don't want to wait to have kids because we want to be young parents.

**PHIL:** I'd actually like to stay at home and let Peggy work after her leave is up. I can set up a home office and be very productive. She has a much higher earning potential than I do.

**ABDUL:** But why wouldn't Sally and Peggy stay at home to care for the children? That's what Kamilah will do. Children need their mothers. That is their work.

**LEADER:** Abdul, your point of view is very interesting and I think probably comes from your cultural upbringing.

**ABDUL:** Most definitely. *(In a rather dogmatic tone)* The role of a woman is to be there for her husband and children. She should ...

**LEADER:** *(In a calm, soft voice)* Abdul, let me interrupt. I think there are obvious differences here based on cultural values. Rather than presenting your view as law, I think it would be better and helpful to hear how your culture views women and children, and then you can hear how Phil, Joe, and others here see the role of women based on their cultural upbringing or desires or what have you.

Here, the leader quickly intervenes when members express differing cultural views and decides that focusing on the differences will be beneficial. The leader also wants to prevent any tension by trying to get Abdul to understand that others in the room have values very different from his.

■ ■ ■

It is the second session of a group of high school senior girls talking about their plans after graduating from high school.

**LEADER:** Where do you plan to go to college?

**MARQUITA:** I'm going to room with my sister who goes to the University of Texas. She loves the university, so I figured why not go there? I want to get away from home but not be by myself.

**SUSAN:** I'm going to go as far away as I can so that my family won't bug me.

**DARLENE:** I'm going to go to a college in Florida because I want to be near the water.

**FATIMA:** I will stay here since I'm not married.

**HEDAYA:** Me too. My father would never support me going away to school.

**SUSAN:** You two ought to live a little. There's nothing here in this town for you.

**MARQUITA:** What in the world does married have to do with it? I think maybe you are afraid....

**LEADER:** *(In a kind, forceful voice)* Hey, hold on just a second. I think we need to consider that what Fatima and Hedaya may be saying may have to do with their culture. Is this right?

**FATIMA:** For sure. I'd love to go away but my father would not think of it unless I was married. It is hard sometimes for me to listen to others in here who have so much more freedom than I do. Is it hard for you, Hedaya?

**HEDAYA:** Yes. I don't think all of you understand our culture.

**LEADER:** Why don't you two take a few minutes and tell us about it?

Here the leader sees the importance of pointing out the cultural differences, and how different values may be operating. Focusing on the differences may clear up some confusion.

■ ■ ■

We believe that in most group situations members should neither be allowed to nor encouraged to argue with one another about cultural or diversity differences. Usually little is accomplished by arguing. This is a group at a mental health center and members are talking about the holidays.

**BRIAN:** ... I think it is wrong that my parents won't let me bring Eric, my lover, home for Thanksgiving.

**RALPH:** I understand why they don't want you to do that. I would not let my child do that. I think it is wrong.

**BILL:** Oh, Ralph, open your mind up!

**RALPH:** What do you mean, open my mind up? You need to read the Bible.

**LEADER:** Wait, guys. We're not having a discussion about homosexuality. We're talking about the holidays. Let's focus on issues you have, and I want people to try to listen and be helpful and understanding. Brian, given your parents' viewpoint, what are you going to do?

In this example, the leader decided not to focus on the differences, because the purpose was to focus on Thanksgiving; it was a week away and the leader knew that a number of the members had some issues regarding the holidays. In a later session, the leader may decide to focus on the different views, especially if the leader feels that the differences are causing tension in the group.

## True Multicultural Groups

As we mentioned earlier, some leaders form multicultural groups or lead groups of people from a different culture than their own. The following examples should give you some ideas about how these groups should and should not be conducted.

### ■ EXAMPLES

The group is a multicultural group on a college campus led by a professor of religious studies from the Middle East and a campus minister. The purpose of the group is to discuss cultural beliefs derived from several religions, and their impact on the members. In recruiting members, the leaders stressed that the intent of the group is more than a simple exchange of information—it will involve personal sharing. Ten students from various cultural backgrounds and religious beliefs volunteered for the five-session group. The members were asked to get into pairs and discuss how their religious and cultural beliefs have been formative in their lives. They are told that when they return to the large group, each will be asked to introduce their partner and talk about the impact of their cultural and religious beliefs on their lives.

**LEADER 1:** Now that we're back, I'd like to start with asking one of you to introduce your partner to the group.

**JENNIE:** I'll start. This is Ashif. He is 19 and comes from Pakistan, where, in his family, he was not allowed to date as a teenager. When he came to this country, his family tried to fix him up with the daughter of a friend from a neighboring village over there, but Ashif saw there were opportunities to meet all kinds of people here and didn't want to date this girl. And this has been a problem between him and his family.

**ASHIF:** This is Jennie. Jennie grew up in what you call the suburbs here. She was into every kind of thing, like soccer and ice-skating and ballet. She feels the suburb where she lived had kind of its own culture, and if you didn't do a lot of things you were not very much liked. She is a Christian but is studying Zen Buddhism now because she wants to make her life more simple and quiet her mind.

**LEADER 2:** That's interesting. Both of you are trying to break from some of the way you were raised. Who will go next?

**TANI:** Yolanda was my partner. She was raised a Christian till she was 12, when her mother married a man who followed Islam, so that's what she believes now. She feels Islam gives her more structure to her life but doesn't like some of the ways that women are treated by Muslim men.

**JENNIE:** *(In an inquisitive voice)* Yolanda, you are not wearing one of those things that cover your head. Why not?

**YOLANDA:** I don't usually dress in with a hijab and that bothers many Muslim men. I have been reading this wonderful book by this liberal-thinking Muslim woman, Asra Nomani, and it has really gotten me to think about courage and how I want to be as a Muslim woman.

**LEADER 2:** Let's finish hearing from everyone and then maybe we can hear more about your thoughts on being a Muslim woman.

**LEADER 1:** *(Having seen Ashif's head nodding as Yolanda was speaking)* Ashif, it'll be interesting to hear what your thoughts are about Yolanda's views.

This is a multicultural group that is going to focus mainly on cultural differences. The leader's role is to facilitate discussion, to help members learn about other cultures, and to challenge their way of seeing the world through just their own eyes.

■ ■ ■

When leading a group made up of people from a different culture or different cultures, you need to understand their culture and ways of being in a group. We close this section with a brief account by one of the authors of how not understanding the culture can really create problems for the leader.

> In 1989, I (Riley Harvill) was fortunate enough to be invited to conduct a series of group counseling workshops as a visiting professor at the University of Hong Kong. Unfortunately I responded to my good fortune with both ignorance and arrogance. I went there thinking that the vast store of knowledge I possessed was going to "WOW" them! After all, they had never—by their own admission—seen live demonstrations of counseling.
>
> Because the participants were eager to see it rather than hear about it, I had no trouble pulling together a demonstration group, and because the participants were well acquainted with each other, we got right down to business.
>
> Touted as a therapy group workshop, one of the women in the group—who had studied in America—quickly shared that she was distant from her mother because of recent arguments, the gist of which centered on old Chinese traditions and family loyalty versus independence and individualism. It seemed evident to me that she wanted to break free of the long-standing cultural bonds of her upbringing and gain the kind of independence that we Americans are so proud of.
>
> If there is such a group-counseling skill as "going for the jugular," I demonstrated it. I was not going to let such a value as desire for independence slip through my therapeutic fingers. Not only was it my fervent intent to help this woman think for herself and break free of archaic obligations, but I was also going to artfully demonstrate to the class how to lead a group member to insight and change.
>
> Like a stage play, the emotional pitch rose in a kind of dramatic crescendo; my client's emotions were palpable as she wept with anguish over the choices that lay before her. The remainder of the group was

transfixed on the counseling work like medical students watching surgery for the first time. I was certain they had never seen anything like this before. I remember thinking that they would certainly anoint me as the next counseling guru, and, when the dissection was complete and my client had been skillfully sewn back together, I looked up with puffed chest only to find that at least one-half the class had left and the other half was considering the same. Furthermore, my group members were looking at the floor as though someone had just died. And although I wasn't sure if the death was mine, the woman's, or the group's, something had gone terribly wrong.

Some time later I discovered that I had caused this poor woman to lose face. I had facilitated not only her crying, but her criticism of her family as well. Furthermore, I had shamed her entire family by encouraging her to reveal the existent riff between all of them. "Her ancestors would not be pleased," someone offered. Great, I thought. I've not only offended this woman but probably her entire lineage extending back to the Xia dynasty. Needless to say, an attempt to offer an explanation of my extremely American-like actions seemed rather trifling in comparison to 3,000 years of shamed relatives.

In the end, it was another one of those dreadful learning experiences and, of course, as these things go, a valuable lesson indeed. Values are powerful and exist within the context of culture, nationality, ethnic practices, and family and life experiences. Not only did I not share her values, I didn't understand them either. The independence that was valuable to me was not valuable to her.

In summary, we believe that effective leaders must give much thought to the individual needs of members. The issues of diversity and multiculturalism you deal with as a leader will be determined by the purpose of your group. At times, these issues will affect the dynamics of your group (Jacobs & Schimmel, 2009). On other occasions, they will be the focus. Although it is unrealistic to expect that group leaders can know every dimension and nuance of a given culture, we hope you will continue to read and discuss these issues, broadening your understanding and awareness and expanding your frame of reference.

## Military Groups

Due to our increased military involvements throughout the world in such missions as Operation Iraqi Freedom (OIF) and Operation Enduring Freedom (OEF), counselors in all types of settings need to have knowledge and skills in working with active duty military, veterans and their families. The psychological impact of fighting a war is being given greater recognition and acceptance along with concerns for spouses and children of deployed and returning soldiers. Spouses are living without their partners for one, two, or more years. Many children are facing living a life where one or two parents are deployed. Many of

these children are being cared for by one parent, or grandparents, other relatives, or friends and neighbors. Family members and returning soldiers have many issues that can be addressed in groups. Counselors in schools, mental health agencies, and other community services facilities need to have programs that can serve soldiers, veterans, their partners, and their children. In this section we give a brief overview of military groups with the hope that anyone who is leading these groups will seek additional information.

## Kinds of Groups

Different kinds of groups can be helpful for military families faced with predeployment, deployment, and reintegration upon returning from deployment. We have divided the kinds of groups regarding veterans and military families into two categories: (1) issues concerning daily living (2) issues related to having experienced direct combat or combat related experiences.

## Daily Living Groups

These groups would include couples' groups before or after deployment, parenting groups during deployment for those with primary childcare duties, parenting groups when the soldier comes home and is reintegrating into the family, job seeking/career groups, and stress management. Counselors run reintegration groups where returning soldiers have a chance to talk about issues that pertain to reintegrating into the community—dealing with friends, family, work, and so forth. More and more the need for drug and alcohol groups with returning veterans is being recognized along with groups that focus on depression, sexual assault, and the threat of suicide. Groups for children with a deployed parent can be run in schools and agencies in locations where large pockets of military families reside.

## Combat Related Groups

Combat related groups include post-traumatic-stress disorder (PTSD) groups; post-Vietnam, OIF, and OEF groups where veterans share their experiences; behavioral reprogramming groups that help members "addicted" to the excitement of a combat zone redirect their energies; and reintegration groups. Returning veterans struggle with unique challenges such as reintegration into civilian world after developing survival skills in a combat environment such as maintaining hypervigilance, which can trigger a sense of danger and alertness in even the most basic functions in civilian life such as driving in traffic or attending social events. Groups can focus on more adaptive behaviors to address the typical avoidance and isolation tactics that these individuals may display and challenge the core beliefs of humanity and justice that they may have. Groups allow these individuals a safe place to share and address these challenges as many service members fear that their thoughts and experiences will upset

others around them. Also, traumatic brain injury (TBI) (impaired cognitive and/or affective functioning) groups are conducted at rehabilitation hospitals and Veterans Affairs (VA) hospitals.

## Screening

Ideally if you decide to lead military groups, you will be able to screen the members. This is important because it is easy to have members who come with an agenda different from the stated purpose of the group. Some may come to focus on their military experiences or issues in their marriage when the purpose of the group may actually be about issues around finding a job or parenting. Unfortunately, many leaders will not have the luxury of screening because they are working at a veterans' center or hospital and the groups are open or mandated. For groups with members with residual TBI or PTSD, leaders will want to screen to determine if the person is functioning at a level where the group experience will be useful and not a source of further frustration and traumatization.

For groups designed to provide veterans a chance to process their war experiences, selecting members who are at different stages of the therapeutic process may be helpful. Members who have shared their stories and have processed the emotional associations sufficiently to move ahead may provide encouragement and support to others in the group.

When leading a group comprised of two or three veterans and the rest of the members being non-veterans, you will want to be sensitive to the fact that what the veterans may be dealing with may be quite different from the non-veterans, especially if their issues pertain to their experiences during combat (i.e., survivor's guilt or the horrors of war). Ideally groups are very homogeneous and consist of either all veterans or veteran's families, but in many settings, this will not be possible.

## Group Size

Therapy groups should probably be limited to four to six members. War processing groups, where members tell of their war experiences, should be limited to 12 with 8–10 being ideal. A group focused on discussing educational opportunities and job finding skills may be larger.

## Length of Sessions

Groups can last from an hour to two hours. In groups where there is intense emotional sharing, the session may last as long as two hours with a break after an hour. The physical health and stamina of your members may also be a factor in session length. Members with traumatic brain injuries may have greater difficulty remaining focused for longer periods of time.

## Number of Sessions

The number of sessions will depend on the purpose of the group. Psychoeducational groups providing information regarding vocational, educational, and career opportunities may be limited to three to five sessions, as may groups intended to provide information about medical conditions, benefits, and so forth. Substance abuse recovery groups, therapy groups, and PTSD groups will continue longer. With closed groups in these categories, it is often better to plan to meet fewer times initially, say eight to ten sessions; then continue the sessions with appropriate members after the initial eight sessions. Some groups will be ongoing with new members cycling in as they enter the system.

## Special Skills

Leaders with an interest in leading these groups should possess good group leadership skills and special training in understanding the military culture, and the stresses of deployment and reintegration. Ideally leaders will have first hand military experience, but groups can be successfully conducted by leaders without combat experience or the experience of being in a military family. These leaders must spend a lot of time talking to veterans and their families and reading about the experiences of military families.

An important skill for any group leader is the ability to handle intense emotions. This may be especially true when leading groups of veterans who are dealing with emotions surrounding the witnessing of the death of their friends and fellow soldiers and/or having the experience of taking another life. Some veterans may experience "flashbacks," that is, emotions so intense that they reexperience the event that has led to their trauma. It is important for the group leader to be able to "stay with" a member who is undergoing a flashback experience and help the member process the emotions or help them to move through whatever it is that they are lashing back to. Ultimately group leaders should become knowledgeable about all aspects that military members, veterans and their families face. This includes bereavement within a family system when a service member dies, military sexual trauma, combat trauma, and struggles coping with changes due to both the physical and invisible wounds of war.

## Comments from a Practicum Student at a Veteran's Center

A few things stand out to me when considering group counseling as it applies to veteran/military population. The first thing is figuring out what type of group it is—educational, support, therapy, etc. I think that these all apply at certain times or for certain topics; however, it can become unclear, which makes it difficult to know how to frame or structure the group. (Adding veterans/military into the examples of the types of groups in the book would be helpful.) The second thing that can make it particularly difficult for someone to work with a group in this population is also something that can happen frequently in other groups as

well—not understanding or clearly stating the purpose of the group. I think a lot of times it is just assumed that a military group involves PTSD. That may be a component of group, but there are a great deal of things that can be involved in a group for military/veterans and this makes the purpose even more important. Many veteran/military groups are organized based on the war that the veterans were a part of—i.e. WWII Veterans Group, Vietnam Veterans Group. That is a great way to form a common basis between members of the group, but it does little in establishing and defining the purpose. A third thing that I would consider to be helpful for counselors in this area would be an understanding that you don't need to be a veteran in order to be helpful. I will liken it to working with folks with addiction—you need to be knowledgeable about it, but you don't need to have been an addict in order to be effective.

I've been picking up on the theme that the groups provide a place for fellowship, and support. For some, it is the one thing they look forward to coming to each week. One of the main purposes of the majority of military/veteran groups is this idea of connection and fellowship. There are a lot of things that come up for veterans that they need assistance with—whether it is job-related, benefits-related, family-related, etc. There is a great need for therapy and working on feelings/emotions, but sometimes the practical help is more of a necessity.

> Another thing that could be further looked into would be the idea of triggers in group counseling. A lot of times in military/veteran groups, the topic of traumatizing events and combat is avoided so as not to trigger other members or be retraumatizing. I would guess that this is also true with other therapy groups as well. Overall, I think that addressing this concept of triggering or retraumatizing group members would be helpful for someone learning how to do therapy groups.
>
> *Angie*

I think identifying the purpose of the group is key. At the Vet Center our groups go on for a long time (10+ years for some Veterans) so it can be difficult for new clinicians to understand because it goes against everything you have ever been taught. However, I think that any clinician/intern that has come through here would agree that as disjointed as it may seem it works and it is helpful to our Veterans. It is about sitting beside someone who truly understands where you have been and what you have seen. In the military you connect to the people in your unit or your squad. At times they are closer to you then even your family. I have watched many of our Vets struggle because when they get out, they lose that connection. However, they feel that connection again when they become a part of one of our groups. As a spouse, I even struggled with that. When Bob got out and we moved back home, I missed my military family. It has been interesting to watch the Vets this winter because they will travel through a snow storm to come to group and I know it isn't because of my stellar personality but it is because they are seeking that safety that comes with being with people who understand.

Understanding the military culture is a big aspect to working with Veterans. I don't believe you have to be a Veteran or have a direct link (it does help,

though) but you do have to have a willingness to learn and to understand. We have an intern now who has done a great job and I think it has to do with them believing that she truly cares about them. Veterans have a way of sensing who is in this business because they want to help and make a difference and those that are working for the government because it pays well.

> I think young clinicians also have to be prepared to hear some pretty horrible things. War is not pretty. Counselors have to be prepared to check their own judgments. I remember as an intern a Veteran shared how his father and fellow soldiers used skulls as bowling balls (this was after the atomic blast at Hiroshima). I remember thinking to myself "how could someone do that?" Well the reality was it was the only way they knew how to cope. Who am I to judge? These were 18, 19, 20 year old men who had been away from home for years and they were exposed to a level of loss of life that I will never understand. This was true in Vietnam (cutting off fingers of Viet Cong as trophies) and Iraq (urinating on Taliban, etc.).
>
> *Tracie* (over 5 years working with veterans)

## Concluding Comments about Special Populations

In this chapter, we discussed considerations and skills involved when leading groups for 11 different client populations: children, adolescents, couples, addicts, the elderly, the disabled, survivors of sexual abuse, those who are divorced, ACOAs, multicultural members, and those involved in the military. Leading groups for each of these populations requires specialized knowledge. Our comments here are meant to introduce you to the issues involved. You will want to do additional reading and attend workshops on the specific groups that you will be leading.

We want to reiterate that there are many populations that we omitted or on which we just touched. For instance, we did not stress specifically gay and lesbian groups, eating disorders, anxiety disorder groups, or women's groups. Each of these, along with so many other populations, could have been covered here and can be explored through books, journals, and the Internet.

## Final Thoughts Regarding Leading Groups

Now that you are at the end of this book, we hope that you feel much more prepared to lead groups and are excited about trying to master the skills presented. At the beginning, we commented that we thought this book would give you an understanding of group dynamics and the skills necessary to allow

you to lead almost any kind of group. We hope the book has been thorough enough to provide you with the basic tools for leading a group. To solidify your learning, we encourage you to review many of the chapters from time to time and watch the video segments multiple times because each time you will see the art of group leading.

No group text would be complete with at least a brief discussion of the future of group work. Most experts seem to agree that group work will continue to be a major force in the field of counseling. Gladding is very excited about the potential for groups: "There is little doubt that in the future, group work will be robust and permeate almost all segments of society" (2012, p. 17). Corey, Corey, and Corey (2014) have listed the increase in short-term structured groups for special populations as one of the major trends of the last decade. We continue to find more professionals seeking training in leading specific kinds of groups where the leader takes much responsibility for what happens in the group. Many school districts are requesting training in group leadership because of the need for many different kinds of groups in the schools. Teachers are also being trained to lead support and academic groups.

The future of group work lies in the integration of counseling theories with an active, multisensory, intrapersonal model of leading. More training in specific group leadership skills is essential if leaders are going to be prepared for the different kinds of groups that will exist in the next 10 years. We believe leaders need to learn more ways to involve the members in the therapeutic process while using counseling theories and the intrapersonal model. Therapists will need and demand better training as they become more aware of the legal and ethical issues surrounding group work.

## Advocacy

Trained group leaders must speak out when untrained helpers are asked to lead groups. Unfortunately, some administrators view group work as something anyone can do, like washing dishes or sweeping a floor. Group work is demanding activity that requires great skill; whenever possible, trained leaders and experts need to keep spreading the word that no one should lead groups unless he or she has the proper training to do so. Far too many groups are led by individuals who have little or no training.

## Social Justice

Social justice has become a major area of emphasis in the field of counseling during the last few years (Singh & Salazar, 2010a). The editor of *The Journal for Specialists in Group Work* in 2010 invited two leaders in the field, Anneliese A. Singh and Carmen F. Salazar, to serve as coeditors for two special issues that are devoted entirely to social justice (Singh & Salazar, 2010a). These two special issues have a wealth of information regarding the history of social justice and examples of social justice applied to group work. One article by Hays, Arredondo, Gladding, and Toporek (2010) addresses the integrating of social justice in group work in the next decade.

## Marketing

Nonvolunteer, reluctant clients make up a large portion of group members in the United States. Residential centers, treatment centers, prisons, and schools often have mandatory groups. Too often, leaders do not show much enthusiasm for leading mandatory groups; thus the members fail to fully benefit from them. Counselors have to do much better at marketing the idea that groups can help people to change. Counselors have to sell this to the members. With nonvolunteer groups, leaders must be excited about what they are doing. In our workshops with the Federal Bureau of Prisons, we are teaching the leaders how to market change in groups with inmates. We have also done this in numerous workshops for drug and alcohol group leaders.

## Groups in Prisons

The prison population in the United States is growing at a tremendous pace, which is forcing prisons to run more groups. Many jobs exist now, and the need for group workers will continue to grow in the prison system because one of the major therapeutic modalities is group counseling. We see this as an emerging area for group workers, and the workers need to be well trained, because the majority of prisoners are not highly motivated.

## Multicultural Groups

Throughout this book, we have mentioned the need to be aware of multicultural issues that may arise during a group session. The recent movement to heighten counselors' awareness surrounding multicultural issues has been valuable (Salazaar, 2009). There now exist many articles pertaining to groups that focus on different cultures. A debate may arise regarding working with groups from the same culture (subculture) versus working with groups from the population at large. Both kinds of groups are valuable. Research will have to answer questions regarding which kind of group is best for various populations. Certainly, groups can serve as a way to gain greater understanding of the diverse population that now exists in North America.

## Closing Comment

We enjoy leading groups and have enjoyed the challenge of writing about what we do. We hope you will review many segments in the book and the videos, especially the integrative segments 17.1 through 17.5 since they pull together many of the skills and techniques described throughout the book. Please contact us if you have any questions, comments, or reactions. We would like to hear from you. (Ed.Jacobs@mail.wvu.edu or for groups in schools, Chris.Schimmel@mail.wvu.edu).